

A GUIDE TO HOMECARE CAREGIVER PREPAREDNESS

A GUIDE TO
Homecare
Dorothy Mooka

*R*REACH
PUBLISHERS

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DEDICATION

I dedicate this book to my visionary late husband, Maitlhoko Mooka, who was instrumental in writing the first chapters of this book, thus helping to make my dream come true.

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My heartfelt appreciation goes to my family for the incredible support they gave me while I was writing this book.

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PREFACE

This is a Guide to Home Caregiving and Caregiver Preparedness

The above statement outlines how prepared and ready the family caregiver is to perform multiple responsibilities required of a caregiver such as:

- Household tasks like paying bills and doing laundry.
- Assisting the care receiver with bathing, dressing and toilet matters.
- Mobility involving moving the care receiver from one point to another.
- Health monitoring such as managing signs and symptoms of disease.
- Emotional and social support.
- Care co-ordination such as navigating and communicating with various healthcare systems.

Family caregiving mainly refers to tasks associated with caring for someone in a private home. Becoming a first-time caregiver can be taxing physically, emotionally and socially. However, transitioning into this new role with greater ease needs the caregiver to have the right support and resources.

In this instance, a caregiving guide is necessary to help the caregiver walk through all that is needed to understand and

know about home caregiving. This process will ensure a successful outcome for the caregiver and care recipient.

A caregiving guide is a road map. It describes, directs, and teaches the caregiver when they are ill-informed, insecure and uncertain regarding caregiving issues. The aim of this guide will ensure a caregiver is prepared to understand topics such as:

- Disease progression and degeneration.
- Medication and medical interventions.
- Knowledge on where to get training for specific conditions.
- Knowledge and skills needed to care for sick persons.

This caregiving guide is written to ultimately equip the caregiver with the necessary knowledge and skills to adapt to healthcare demands. This will lighten the difficult caregiving path and improve their preparation and readiness through education and guidance in home care issues.

It does not matter at what stage you are at in your caregiving journey, this guide will help you get ready to navigate your role as a caregiver – someone who learns about what caregiving entails, issues like death, funerals and how to plan to care and find solutions.

A prepared caregiver will be associated with positive emotions such as self-confidence, self-esteem, self-efficiency, coping skills, and well-being.

This guide has been developed because of the author's deep-seated desire to assist those who are beginners to caregiving and to help them along this almost impossible path.

The author is a qualified nurse educator, who has specialised in community health nursing. She has vast experience and has worked in health training institutions, hospitals, community health settings in rural and urban areas. Her responsibilities, among others, have been to supervise students develop community health projects and conduct home visits. This has become the main motivator to write this guide – a guide to provide information that is simple and clear to understand.

Many caregiving guides are full of professional jargon that students and family members struggle to understand. The author was determined to write a simplified guide without losing the importance of the caregiving process.

How to Use this Guide

This caregiving guide is organised into 12 chapters, each one covering a different topic, and each one translating into a step-by-step process of becoming a caregiver. The reader

is at liberty to choose the order of reading the chapters as determined by his/her level of previous preparation in home caregiving.

It is recommended that new caregivers read all chapters to be better prepared, while experienced caregivers will find the guide to be a valuable source of reference material.

The challenges faced in writing this book included the availability of information covering many platforms and condensing the information to be relevant to people at different levels of readiness in home caregiving.

Common Concepts in this Guide

Health: This is a state of complete physical, mental, social, and spiritual well-being.

Wellness: A state of being in good health, becoming aware of yourself and making choices to live your life to the fullest.

Autonomy: Making independent decisions and choices.

Assessment: Activities performed by a caregiver with at least one health professional (nurse or social worker) to determine current ability to function in six areas: physical health, social

support activities of daily living, environmental activities, financial situations, health monitoring, and co-ordinating care.

Everyday Tasks: Basic personal tasks that a person does independently and regularly – such as bathing, dressing, grooming, homemaking and work.

Self-care: This is the ability to take care of oneself in a way that promotes health and active management of illness when it happens.

Self-care Deficits: This is the inability to perform certain functions related to health and well-being such as bathing, dressing and eating.

Caregiving: Is all about the identification of problems and solving them.

Care Recipient: An individual with a medical condition or who requires support with everyday tasks.

Family Caregiver: A caregiver is a family member, friend, close relative, or community member who has taken on the responsibility of physical duties, emotional needs or financial affairs of a person who cannot take care of him/herself because of age, illness or disability.

Family: A group of persons related by blood as well as interaction, such as parents, children, uncles, aunts, cousins, grandparents, friends, neighbours and health professionals.

Community: Is made up of a group of people who share common interests such as place, religion, norms, values, beliefs and identity.

Family Health and Wellness: Family health is a state in which the family is a resource to its members as far as daily living activities are concerned. The family is also responsible for supporting its members.

Caregiver Preparedness: Preparing caregivers to perform tasks, duties and responsibilities to individuals who are sick at home.

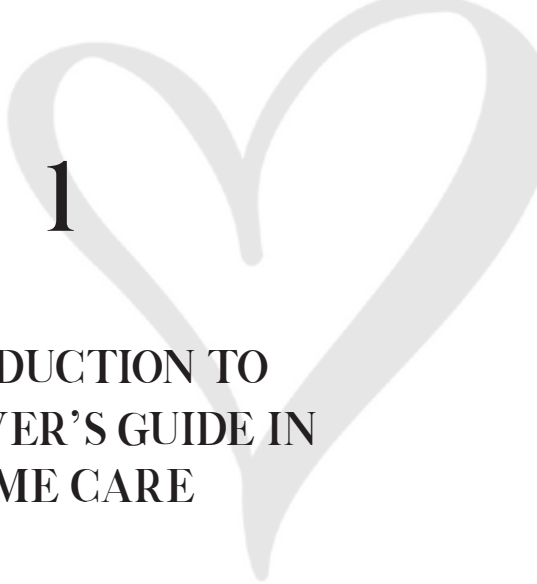
Ill Health: Is an unhealthy condition of the body and mind, or a general term used to refer to the physical illness or disease caused by germs.

Caregiver Burden: The physical, emotional, social and financial toll of providing care.

Assistive Technology Devices: Products used to improve a person's ability to function independently – such as a motorised wheelchair, walker, hearing aid, pill organiser and bath seat.

Continence: The ability to control bowel and bladder functions.

Palliative Care: Total care of a person with a terminal illness. The focus is on quality of life. Control of pain and other physical, psychological, social and spiritual symptoms.



1

INTRODUCTION TO CAREGIVER'S GUIDE IN HOME CARE

Caregiving is something everyone in their life has to face. And all people need to identify their level of caregiving readiness to adopt the caregiving role.

Why is a Caregiver's Guide Essential in Home Care?

- A family home caregiving guide is intended to prepare those who provide care to individuals at home who are not able to look after themselves.
- A beginner in caregiving lacks the appropriate knowledge and skills to provide quality care. It is for this reason that a caregiving guide exists to lighten the difficult path.
- The caregiver's guide provides information that is simple and clear to understand, thus improving the quality of care provided.

- The caregiver's guide begins with the basics of caregiving and will carry a novice through all the stages of caregiving.
- The caregiver learns to make safe decisions comfortably.
- The caregiver learns to provide quality care through everyday tasks such as bathing, dressing, toilet care, walking and meal preparation.
- The guide empowers the caregiver to execute duties with confidence.

What is Home Caregiving?

- Home caregiving is the art of providing care to individuals in their homes who cannot care for themselves, either physically, emotionally, socially or spiritually.
- Home care services may be provided on a visiting or live-in basis.
- The care is performed at home by family members, neighbours, friends, or volunteers in the community.
- Recipients of home care are the aged, disabled, acute and chronically ill persons.

Home Care is Preferred for the Following Reasons:

- It becomes an optimal solution for those who need assistance on a regular basis.
- The aged, disabled and chronically ill seem to recover better at home than in institutions.

- The care receiver at home is in control of the situation.
- Care recipients maintain a sense of independence while at home.

The Basics of Home Caregiving are as Follows:

- Giving support and encouragement to patient and family.
- Assisting with patient medications.
- Managing symptoms and side effects.
- Arranging for patient appointments.
- Performing house chores and payment of bills.
- Sorting legal issues, e.g. wills and meals.
- Assisting with financial problems and title deeds.

Caregivers Have Multiple Roles. They Can also be:

- Spouses.
- Housekeepers.
- Professionals.
- Parents.
- Family members.

How Does Home Caregiving Start?

- Caregiving can sneak up on you or start suddenly.
- It can start when you check on a relative and help by doing small tasks with house chores, etc.
- Refilling prescriptions.
- You notice unfinished housework.
- Bills are not paid.
- Poor personal hygiene.
- Soon you feel committed to care full-time for the relative.

Caregiving Could also be Triggered by the Following:

- Major event, e.g. stroke or accident.
- Aging process that contributes to memory loss and forgetfulness.
- Chronic illness.
- Disability.

At this stage the caregiver is anxious to know how to assist with home care.

What Information is Necessary for the Caregiver to Provide Home Care?

Assess the Caregiving Situation as Follows:

- Form a caregiving team made up of care receiver, family, friends, etc.
- Talk to the care receiver and family members.
- Collect personal details and past and present medical history of the client.
- Address present and future problems or needs.
- Respect the preferences, wants, wishes and values of care receivers.
- Review home environment, all areas of daily living, and health status.

Get an Accurate Diagnosis from a Medical Doctor as Indicated Below:

- Know about medical condition/s – signs and symptoms.
- Treatments, side effects and interventions.
- Discuss treatment options and level of care needed.
- Supervise treatment.

Determine the Care Receiver's Needs, Considering the Following Pointers:

- Need for personal care – like bathing, toilet needs, feeding.
- Household care – cleaning and keeping the house in order.
- Healthcare such as medication and appointments.
- Companionship, meaningful activities and conversation.
- Supervision – oversight for safety at home to prevent falls and wandering, especially with the elderly.
- Learn appropriate knowledge and practical skills necessary for caring for the client.
- Know where to get trained to do caregiving tasks such as feeding, toileting and incontinence, bathing and administering complicated medication.

Plan How to Care

- Set long and short-term goals of care.
- Think of who will do each task.
- Determine roles and assign them.
- Match caregiving expertise from team members to the patient's needs/problems.
- List problems identified.
- State how you will solve the problems identified.
- State who will help you to care for the patient.
- Write schedules and timetables.
- Take inventory of all resources at your disposal.

What does a Family Caregiver do?

- A caregiver cares for a family member who has self-care deficits or is unable to look after himself.
- Operates as an extension of the healthcare system.
- Needs to navigate the social services system.
- Performs complex medical and therapeutic procedures.
- Ensures care recipient adheres to therapeutic regimes.
- Functions as a home-based care co-ordinator.
- Advocates for the care recipient.
- Learns how to perform activities of daily living.
- Considers who the best person is to do each task.
- Determines roles and assigns them.
- Sets short and long-term goals of care.

Caregiving cannot be done in isolation. The caregiver needs other people to help who are members of the caregiving team. People like:

- Close and distant family members.
- Neighbours.
- Friends.
- Health professionals.

Common Caregiver Duties and Responsibilities Include Assisting Clients with Everyday Tasks Such as:

- Personal care – bathing, grooming, dressing, toileting, exercising.

- Basic food preparation – preparing meals.
- Shopping, housekeeping, laundry and other errands.
- Appointment reminder of medication.
- General healthcare – overseeing medication, prescription usage and administration.
- Mobility – getting in and out of wheelchair, car, and shower.

Below is the Process to Empower the Family Caregiver:

- Talk to health professionals on issues that bother or concern you.
- Read about the disease progression of the patient and caregiving in this situation.
- Learn about the level of care needed and the type of resources available to help.
- Do research – good sources are the internet, library, e-books, videos, YouTube, conferences, meetings, workshops, or even enrolling for short courses.
- Keep the information in a folder.
- Knowledge will increase your confidence and reduce anxiety.
- Use a smartphone or tablet to easily access knowledge anywhere.

No Matter what the Caregiving Situation is, Remember the Following Guidelines to Preserve Dignity:

- Consider the desires of the care recipient as he/she has the right to run his/her life.
- Take heed not to ignore wishes, concerns, and wants or preferences of the care recipient.
- Provide privacy where necessary – during bathing and dressing assist only if the care recipient is unable to care for themselves. Observe independence at all times.
- Patience is crucial as the care recipient is likely to feel frustrated and irritated.
- Allow the care recipient to make decisions on basic tasks like what to eat, what to wear and what bills should be paid. Encourage independence.
- Promote independence and only help if the patient has been given a chance to do it themselves; otherwise self-esteem can be destroyed.
- Ask for assistance from family, friends, church members and neighbours so that there is interaction with different individuals.
- Be an advocate for the care recipient everywhere, particularly in healthcare facilities.

Characteristics of Caregivers:

- Empathy.
- Compassion.
- Patience.
- Trustworthy.
- Caring.
- Objective.
- Dependable.
- Respectable.
- Observant.

**The Caregiver Needs to Understand the Challenges of
Caregiving as Discussed Below:**

1. Time Management:

- Involves accommodating all tasks and having time for self-care.
- Caring, doing family work, having an outside job.
- Ability to participate in community responsibilities.
- Failure to balance your time results in stress.

2. Stress:

Physical signs:

- Lethargy.
- Headache.
- Chest pains.

- Gastric pain.
- Insomnia.

Emotional signs:

- Low self-esteem.
- Anger.
- Anxiety.
- Irritability.

3. Lack of Privacy Due to:

- Staying in the same household with the patient.
- No time to rest.
- Working long hours.

4. Lack of Sleep Due to:

- Working long hours not scheduled, especially as a stay-in caregiver.
- Expectations not clearly defined and adhered to.

Caregiver Benefits of Caring for Others:

- Feeling good and needed, having a greater purpose in life after helping someone, focusing on more important things.

- Feeling a sense of accomplishment as a result of being useful to someone and bringing joy.
- Feeling valued and building a strong relationship.
- Caring for others makes one have empathy and the ability to connect with others. The benefits outweigh the challenges.

The Success of a Home Caregiver Depends on the Ability to

- Work without supervision.
- Get along with patients and others.
- Organise a time to take care of yourself.
- Provide home care.

The Biggest Challenge for a Primary Caregiver is

- Being the main person handling the everyday tasks of a sick relative.
- Being expected to be empowered with up-to-date caregiving information about the care recipient's condition.
- Being an advocate and making the best decisions on behalf of the care recipient.
- Finding time and energy to take care of your own health.
- Many caregivers suffer from depression and anxiety and may delay regular visits to their doctor.

- Being a primary caregiver often compromises the caregiver's own well-being.
- The caregiver is overwhelmed, exhausted, overworked, and feels guilty about caring for themselves.
- Care receiver becomes the priority and caregiver neglects own healthcare needs.

The Following are the Tasks a Caregiver Needs to Perform to Stay Fit and Healthy

- Exercise for 30 minutes a day.
- Eat a balanced diet three times a day consisting of whole grains, fruit and vegetables, and protein.
- Sleep eight hours a day.
- Have annual medical examinations.
- Have rest time from working to allow the body to recharge.
- Take breaks from caregiving to recharge.
- Spiritual self-care is important.

Find Government/Local Services

Locate Caregiver Support Services

- Learn about home and community care support in your area.
- Find family support.

- Find out where home care support is available from, e.g. agencies, elder care.
- Support groups are good for exchanging information.
- Share advice on caregiving – now and in the future.
- Give lists of volunteers and resources.
- Learn from support groups.
- Search online for transportation, skilled nursing support, e.g. training complex caregiver skills.
- Locate caregiver supportive social networks.

Take Advantage of Community Services Available Such as

- Day-care centres – this refers to home aids and home meals.
- Respite care – where to look for assistance to relieve you of your caregiver tasks.
- Call senior centres for information – services, referrals, social workers and advocacy groups.
- Personal care services – help with bathing, dressing, feeding, meal preparation and companionship.
- Healthcare services – provided by trained health professionals.

Community Nurses, Occupational Therapists and Social Workers May

- Assist to make recommendations to modify the caregiver's house to make it suitable for caregiving.

- Assist to make a choice of equipment, devices and aids.
- Get assistant caregivers and respite care.
- Some hospitals offer classes to teach caregiving skills needed for your situation.
- Ask your doctor for referrals or call a local agency for guidance.

Respite Care Providers Perform the Following Function

- Drive patient when going to an appointment.
- Meal planning and cooking.
- Extra help around the house.
- Laundry and doing errands.
- Unplanned doctors' visits.
- Plan a holiday with relief from the agency.

2

SELF-CARE AND EVERYDAY TASKS

In this chapter the caregiver learns about self-care and everyday basic and essential tasks for the survival of individuals, families and communities.

Katz, S, (1983) describes how to assess self-maintenance and activities of daily living that everyone needs to function independently.

Self-Care is a Set of Behaviours and Tasks Performed Daily by Individuals to

- Promote health.
- Prevent disease.
- Maintain health.
- Cope with illness with or without a care provider.

What are Everyday Tasks?

Below are statements that describe everyday tasks.

- A collective term used for fundamental skills to enable individuals to function independently.
- A series of actions that are automatic and routine.
- Basic self-care tasks learnt in childhood.
- Tasks taken for granted and done every day without assistance.

Everyday Tasks Consist of Basic Self-Care Tasks that Include the Following

- Personal hygiene – including bathing/showering, grooming, nail care and oral care.
- Dressing – deals with being able to select appropriate clothing to wear as well as to dress and undress oneself.
- Eating – addresses the ability to take food from the plate to the mouth without spilling and then swallow it.
- Maintaining continence – the ability to use the toilet and clean oneself without any help.
- Transferring/mobility – this refers to the ability to move oneself from point A to point B without any assistance.
- Basic communication skills – the ability to regularly use the phone, email and internet and hold a simple conversation.
- Transportation – the ability to drive oneself or use public transportation.

- Meal preparation – planning, cooking, cleaning up, storage, ability to safely use kitchen equipment and utensils.
- Shopping – the ability to make appropriate decisions regarding buying clothing and groceries.
- Housework – household chores like doing the laundry, dishwashing, dusting, vacuuming, gardening, etc.
- Managing medications – ensuring that client has medication and adheres to treatment regime as prescribed by a physician, e.g. taking the correct dosage, time, and amount.
- Managing finances – the ability to pay bills, write cheques, use an ATM machine, etc.

**The Difficulty in Performing Everyday Tasks
Could be Because of:**

- Injury.
- Ageing.
- Disability.
- Disease that is acute or chronic.

**Caregivers are to Assess Everyday Tasks to
Achieve the Following:**

- Determining the care receiver's self-care deficits or inability to accomplish essential tasks.

- Identifying self-care or the ability to perform everyday tasks independently.
- Determining total dependence, in other words, when the individual cannot do anything for themselves.
- Knowing what kind of skills and resources are needed to support the care receiver.
- Assess for care receiver's ability to safely and completely do these tasks without direction, supervision or assistance.
- Assess the ability to do these tasks and determine a care plan.

Signs and Symptoms of Self-Care Deficit Include:

- Bathing – the inability to get out of bed to take a bath/shower, or to wash oneself and get back to bed.
- Grooming – unkempt hair, not washed or combed. Finger and toenails not trimmed and cleaned.
- Dressing – has trouble removing clothes and putting them on again, wearing shoes and socks. Clothes worn for long periods with bad odour.
- Toileting – the inability to sit and stand from the toilet seat, without help, and not able to clean oneself after using the toilet.
- Eating – needs to be fed, not able to bring food from plate to mouth.

- Transferring/mobility – has trouble moving from a bed to a chair or walking around the house alone.
- Unexplained burns – this would suggest problems with cooking.

Caregiver Observations Must be Communicated to the Care Receiver as Follows:

- Engage in open, compassionate discussion.
- Determine the best strategy to deal with family members.

How Does the Caregiver Solve Problems?

- The answer will be determined by the living conditions, preferences, and challenges each person is confronted with.
- Minor adjustments can solve problems, like having dresses with zippers at the front instead of the back.
- If there's a problem with cooking meals, have precooked meals delivered.

**Caregiver Intervention for Self-Care Deficits
Includes the Following:**

- Observe self-care deficit task, e.g. toileting.
- Support patient and family.

- Act for and do for the patient.
- Promote activity involvement and functional independence.
- Involve patient in achieving developmental milestones.
- Establish the reason for patient impairment, e.g. surgery recovery, mental health issues.
- Seek appropriate assistance.
- Encourage the patient to verbalise thoughts, fears, anxieties and frustrations.
- Rehabilitate, guide and teach self-care.

3

GETTING READY FOR THE ROLE OF HOME CAREGIVER

Scherbring, M, (2002) asserts that caregiver readiness is associated with positive emotions such as self-confidence, self-esteem, self-efficiency, coping skills and well-being.

The caregiver has to start the caregiving journey by identifying the health problems of the relative and determining how care will be provided now and in the future.

The Caregiver Spots Something is Wrong with the Relative Under the Following Circumstances:

- Notices things likely to cause falls in the house.
- Looks like unfinished business in the house, e.g. dirty dishes, mail unopened, laundry not done and the house unkempt.
- Car has unexplained dents.

- Missing meetings, appointments, obligations, forgetfulness, e.g. Alzheimer's, dementia.
- Difficulty in performing routine daily activities.
- Difficulty in sitting or standing on his or her own – could be the cause of falls.
- Unexplained bruises or wounds.
- Poor housekeeping.
- Weight loss.
- Poor medication management.
- Difficulty with mobility.
- Frequent car accidents.
- Poor personal hygiene, e.g. unkempt, not bathing.
- Confusion.
- Incontinence.

What to do Next:

- Determine the level of care needed.
- Find out the number of hours care would be needed.
- Look for options.
- Decide if you want to be a caregiver or not.
- Consider hiring a private caregiver if you cannot cope with daily responsibilities.
- Consider leaving your employment.
- What are the consequences in monetary terms?
- How will bills be paid?
- The degree of involvement to be determined by the stage of disease.

The Caregiver Prepares to Make the Transition into Caregiving Through the Following:

- Dropping by, visiting or staying with a client.
- Assisting with housekeeping.
- Taking a client to doctor's appointments.
- Buying groceries and preparing meals.
- As duties increase, the caregiver becomes overwhelmed.
- Accepting you have a challenge in the new role of caring for a sick relative.
- Starting to plan for the challenge as the care receiver feels very lonely.
- Seeking support from family members and the community.
- Taking charge without taking over.
- Allowing clients to do things for themselves.
- Letting them make decisions.
- Preserving their independence.
- Do not treat them like children.

How the Caregiver Prepares to Talk to the Care Recipient as One who has the Biggest Say in Caregiving:

- Talk to care recipient, tactfully introducing caregiving as a subject and saying how you could assist.
- Indicate interest to increase the understanding, knowledge and skills of family caregiving.
- Collect personal medical history of the patient.

- Find out needs, problems, concerns and preferences of the patient. Involve patient in the decision-making, and where caregiving will take place – at own home or nursing home.
- Discuss financial issues like who pays for the caregiver.
- Discuss legal issues like who will make the decisions when the care receiver is mentally unstable.
- Care recipients might initially be reluctant to participate. Pursue the topic until she agrees to discuss her care planning at home.

How Caregiver Becomes Ready to Identify a Client's Needs and Goals:

- Ask what is lacking in the client's routines.
- What objectives do you want to achieve?
- Based on the above results, your assessment is complete.
- List all the shortcomings, concerns, preferences, wishes or problems.
- List needs/problems according to priority – life-threatening issues are the most important.

What to do to be a Successful Caregiver

The following are tips for preparing yourself for the challenges of caring, like priorities and realistic goals:

- Make lists of what needs to be done, putting the most important first.
- Arrange your day using any help available.
- Develop a schedule to cover the entire day.
- Do not overload yourself.
- Form partnerships with the care receiver, family and healthcare providers.

Getting Ready to Care as a First-Time Caregiver, Make Sure you do the Following:

- Participate in a discharge plan before the patient leaves the hospital to be cared for at home.
- The doctor will give a medical diagnosis, treatment and other interventions and side effects.
- The social worker will advise on community resources – rehabilitation programmes, respite care, community hospital dementia day-care, home help services, and caregiver support services.
- Educate yourself and your family caregiving team for the following reasons:
- Information is power; therefore learn all about the client's condition, progress, treatments and medications.

- The caregiver's priority is to get training and learn new caregiving skills. Do this by searching the internet, watching videos, reading books, and attending workshops, conferences and meetings.
- This will help you solve problems now and in the future.
- You will familiarise yourself with care options such as assisted living or home care.
- Get training from health professionals on complex procedures like giving injections, putting on dressings, and checking blood pressure.

How to Establish a Support Team?

Choose representatives from the following groups:

- Close and distant family members.
- Your church members.
- Caregiver support groups.
- National caregiver organisations.
- Organisation/s specific to illness or disability.
- Therapists, social workers, counsellors or nurses.

Identify the Expertise of Each Team Member and Assign Tasks as Appropriate:

- Delegate caregiving tasks to team members.
- Make a list of viable solutions to problems in a care plan.
- Assign tasks according to the expertise of the team members.

Understanding Caregiver Role

The caregiver role begins when a medical condition is diagnosed. The caregiver needs to know the following to be able to understand how to care for the care recipient:

- Learn about the signs and symptoms of the condition.
- Understand how the condition progresses, treatment, and medical interventions.
- Ways to manage the condition.

The Caregiver Decides Where Accommodation will be and Must Consider the Following:

- Care receiver will continue living in their house, move in with the caregiver, or move to some other place.
- Affordability and sustainability.
- Does the health condition allow for the person to live alone?
- Is living with the caregiver a viable option, e.g. lack of privacy and control of boundaries?
- Discuss what to do and what not to do.

Determine Accommodation Options Appropriate for the Sick Relative Such as:

- Assisted living homes.
- Retirement homes.

- Senior housing care.
- Transition home care.
- Hospice supportive homes.
- Respite care centres.

How to Find a Home for a Chronically Ill Relative:

- Choose the best options.
- If staying at home is the best option, then choose home care.
- Select the best agency for the best outcomes for assisting persons with everyday tasks.

How to Prepare a Room for Caregiving if Staying in the Same House:

- Choose the room to be used by the care recipient.
- The room must be safe and accessible.
- The house must be free from distractions.
- Preferably choose the ground floor without stairs and near a kitchen and bathroom with support bars, ramps for wheelchairs, etc.

In Home Modification, Consider the Following:

- Make sure your home is accessible.
- Coming in and going out is made easy.

- Build a wheelchair ramp, if necessary.
- Construct wider hallways and doorways.
- Clear spaces for a wheelchair.

Kitchen Considerations

- Lowering kitchen cabinets, counters and sinks.
- Replacing cabinet knobs with pull bars.

Bathrooms Considerations

- Installing grab bars in the bath.
- Getting rubber grips for faucets.
- Regulating water temperature to avoid burns.

Passages and Stairs

- Add night lights and increase the wattage.
- Ensure stairs are well lit.
- Be cautious of throw-rugs that pose a slippery hazard.
- Install slip-resistant floors.
- Install electric outlets that can be easily reached.
- Use smart door locks, where the phone is used to lock and unlock doors.
- Lights can be used by remote control or sensor movements.

In Order to Make your Home Safe, Attend to the Following:

- Take care of safety and possible dangerous situations.
- Consider sharp objects like knives and scissors.
- Consider poisons, medicines and dangerous household substances.
- Garden tools in the house.
- Watch out for fire hazards such as cigarette lighters, stoves and matches.

Make Sure of the Following:

- There is an emergency exit, as well as locks to secure the house, and alarms.
- Emergency phone numbers are available.

List What Needs to be Achieved and How to do Your Plan, Such as:

- Have an organised approach to make sure you have things in control.
- Create backup plans and ask others to serve as backup reinforcement.
- This reduces stress and ensures the patient gets the necessary help. Write plans and schedules to introduce the care plan.

A Caregiver is Guaranteed to Succeed Where There is:

- Careful planning and good self-care.
- Knowledge and resources available.
- Caregiver support groups.

How to Prepare to be a Home Caregiver:

- Research and learn as much as possible about the family member's disease.
- If necessary, enquire from the care recipient about their financial and legal status.
- Get in contact with the care recipient's doctor and other health professionals that use the same insurance.
- Collect legal information such as wills, as well as power of attorney in preparation for when the patient is unable to make decisions.

Find Out About the Client's Finances Such as:

- Determine expenditure and available income.
- Determine financial needs.
- Find out how bills will be paid.
- Consult a financial planner.
- Identify the financial needs of the care receiver.

Identify Sources of Income to Pay for Care. For Instance:

- Look for public benefits.
- Consider medical aid schemes.
- Learn about the financial situation of the sick person – do they have any insurance?
- Are there any insurance policies?
- What about pensions and employee allowances?
- Worker's compensation.
- What are the care recipient's entitlements?
- Identify available options.

Talk About Legal Documents Such as:

- Birth certificates.
- Marriage certificates.
- Property documents.
- Wills, assets and liabilities.

Find Out About Available Community Resources to Help:

- Select from available national and local resources to make your caregiving job easier.
- Arrange for an adult day-care centre, respite care, services such as Meals on Wheels, and limited financial assistance.
- Religious organisations – churches, temples or other places of worship.

- Support groups, local hospitals and online support groups.
- National caregiver organisations, social welfare workers, counsellors.
- Get other family members to take part in the caregiving.
- Contact the elderly network to solicit employee benefit assistance. This is to also assist with time off from work.

How to Care for Your Loved One

How to help your loved one adjust to loss of independence due to illness.

- Enquire about where you can find information, resources, and tools that can make your caregiving task easier.
- Advocate for self and care receiver.
- Ensure the home environment is safe and secure.
- Install burglar bars, adjustable shower, night lights and grab bars, as necessary.

4

GETTING ORGANISED IN CAREGIVING

This section will help the caregiver prepare and organise important documents and medical information to become an effective caregiver.

Be Organised as a Caregiver for the Following Reasons:

- So as not to bring chaos to your life and that of the care recipient.
- To have a sense of control and be able to think through decisions.
- To have legal documents and a financial plan in place.
- To be able to keep track of overwhelming information on medication, medical appointments and important telephone numbers.
- Instead of relying on lists, use systems to organise data like the Microsoft HealthVault.

According to Bursack, C, (2020), caregivers are different and are capable of organising caregiving differently in order to achieve desired objectives.

Below are various ideas caregivers might want to adopt to get organised while caregiving.

Getting Organised in Caregiving by Establishing Routines

A routine is all about doing the same basic activity at the same time every day:

- When a daily routine is established, the caregiver and family follow a set way of doing things, e.g. patient wakes up 8am, uses toilet, brushes teeth, baths, combs hair, gets dressed.
- At breakfast, create a daily schedule for taking medication.
- Activities can be scheduled daily, weekly or when necessary, so that doing things happens naturally throughout the day. This increases the feeling of security and independence.

Caregivers Set Up Daily Schedules to Perform Tasks Such as:

- Attending doctor's appointments.
- Collecting prescriptions.
- Paying bills.
- Doing shopping. It is only through setting schedules that order is established in caregiving.

Here is an Idea of a Suggested Daily Schedule

1. Morning Schedule

The day proceeds as follows:

The patient wakes up at an agreed time. As this is the time of day when the care recipient has the most energy, the caregiver must encourage independence and only assist with activities the care recipient is unable to do on their own.

Bathing frequency – shower or bath once or twice a day to keep the skin healthy. Where not possible, use a hand basin and small towel to wipe the face, arm pits, genitals, groin, feet or any other folds to remove bad body odour.

Basic personal hygiene – oral care follows, like brushing teeth or dentures. Moisturise the skin with ointment or lotion of your choice. Select appropriate clothes for the day.

Daily breakfast – this should be balanced and as preferred by the care recipient.

After breakfast, and depending on the individual, the following activities could be done – tidying up around the home; reading the newspaper; cup of tea; playing favourite music; a bit of gardening; leisure walks, etc.

2. Afternoon Schedule

Prepare lunch, discuss meals with the care recipient, and where possible, allow participation in food preparation.

Caregiver assists with cleaning up, washing dishes, visiting a friend, going for a walk, light exercise, finishing home chores and attending doctor's appointments.

Leisure time could include watching a movie, doing quizzes and other games, as well as time to rest.

3. Evening Schedule

Eat dinner two to three hours before going to sleep for good digestion.

Meals should be light to aid good sleep; stimulants are not recommended.

The caregiver tidies the home before bedtime.

Before bedtime, these activities should happen – a quiet time, relaxing activities like music, games, a warm bath, casual conversation.

Develop Caregiver Calendar

- Keep the planning calendar coded to include all important events and activities.
- Keep a second calendar for writing specific help from others such as specific tasks, e.g. care team calendar, care calendar, community calendar.

How to Set Up a Caregiving Calendar for Family

- Since you cannot do caregiving all by yourself, involve the caregiving team.
- Make a list of all specific caregiving duties with each task such as:
 - Rides to doctor's appointments.
 - Assistance with personal care.
 - Shopping and groceries.
 - Meal preparation.
 - Medication schedules.
 - Household chores, laundry, etc.
- Send list to all members of the caregiving team.
- Contact team members to choose what they can help with or are unable to do.

- Helpful calendars found on the internet include: Lotsa Helping Hand, Google calendar and Care calendar.
- Include long-distance family members who can help with hands-off care such as:
 - Managing finances like paying bills, budgeting and searching for an appropriate institution where the care receiver can stay.
 - Information co-ordinator who does research on health issues, mediates, advocates, sorts insurance coverage.

Provide Essential Contact Details for:

- Family, friends and significant others.
- Doctor and other health personnel.
- Insurance companies.
- Lawyers.
- Financial advisors.
- Emergency contacts are to be displayed in a public place that is easy to find.

Make a List of Service Providers Such as:

- Home health providers.
- Cleaners.
- Gardeners.
- Handyman.

- Any other people who may provide help around the house.

Make a List of Caregiving Tasks for Use by Someone When The Caregiver is Not Available. Examples are:

- What the patient prefers to eat.
- Where to find the blood pressure machine.
- Caregiver needs to put toothpaste on the patient's toothbrush.

Create a System for Medication Management

The list should include:

- Information, emergency numbers, health records, prescriptions.
- Plan when to collect drugs from the pharmacy.
- Store medications in order; do not confuse them. Ensure they are well labelled.
- List daily, weekly and monthly routines you carry out.
- The person who ordered medication, for what, dosage, frequency and from what pharmacy.
- Draw doctor's attention to dietary supplements that may interact with drugs ordered.
- Use a pill organiser to store drugs safely.
- Monitor taking of drugs.
- Remove expired drugs.

- Set up a reminder to take drugs.
- Keep extra supplies for restocking, e.g. over-the-counter medicines.

Keep a Notebook for Observations Such as:

- Condition of patient's physical and mental welfare and other things that happen once in a while.
- Find a relief caregiver in case you are not always available.

Identify Community-Based Services

- Determine what private and public programmes are available.
- Find out what the less privileged can get for free.
- Find home care agencies.
- Find respite services.
- Find policies that exist.
- Purchase assistive devices for home use to avoid hospitalisation.
- Purchase home care continence products and supplies to make adult daily caregiving activities more convenient.

Organising your Documents

- Collect important documents and put them in a place where they will be easily retrieved when necessary.
- Insurance information.
- Banking information.
- Information like wills and assets.

How to Use Digital Organisation in Caregiving

There are various apps available to help caregivers be on top of situations. Consider the following:

- Use online organisation tools such as Google Drive or DropBox to create a virtual binder of medical and financial documents.
- Create an individual file for each family member.
- Medical reminders – CareZone is a free app that allows medication reminders to be set, track appointments, and order refills all in one place.
- Med Safety is an app for medication, schedules and reminders that alert one if medication is missed.
- Group app – Caring Bridge and Caring Village can be used for groups.
- Monitoring – Amazon's Alexi device can offer Care Hub, etc.

5

KNOWLEDGE AND SKILLS NEEDED BY CAREGIVERS

In this chapter, a caregiver gets prepared and ready by learning more knowledge and skills in the caregiving role. Given, B, Sherwood, P, Given, C, (2008) argue that quality care provided by caregiver is determined by knowledge and skills acquired.

Why Knowledge and Skills is Required in Caregiving

- The family caregiver must acquire knowledge of the disease, prognosis, care plans and treatment protocols needed.
- This lays the foundation for teaching a caregiver complex tasks and skills necessary to handle illness at home.
- The skills will instil confidence and competence in the caregiver.

How to Acquire the Knowledge Needed by Caregivers to Provide Quality Care

First, determine the area of care in your caregiving role such as:

- Household duties – cleaning, dusting and helping to manage money.
- General hygiene such as bathing, toileting and dressing.
- Managing medical care that includes administration of medication and maintaining of records.
- Scheduling of care appointments with healthcare providers and support services on behalf of the client and other parties.
- Liaising with care providers and support services on behalf of the sick person in all situations.
- Co-ordinating or providing necessary transport for the client to attend appointments, shop, etc.
- Listening to and advocating for the client.
- Knowledge acquired must be relevant to the needs of the client.

Knowledge Will Enable the Caregiver to Achieve the Following:

- Observe changing condition of the patient, be able to monitor signs and symptoms and act appropriately as trained.
- Interpret what signs and symptoms mean.

- Make appropriate decisions to render safe care.
- Take action as needed.
- Make adjustments to changing needs.
- Access resources to benefit the care receiver and caregiver.
- Provide hands-on care that will ensure quality of life.
- Work together with the patient.
- Navigate the healthcare system to benefit the client and caregiver.

Below is the Process to Empower the Family Caregiver:

- Talk to health professionals on issues that bother you.
- Read about disease progression of the patient and its requisite caregiving.
- Learn about the level of care needed and the type of resources available to help.
- Do research – internet, library, e-books, videos, YouTube, conferences, meetings, workshops.
- Keep the information in a folder.
- Knowledge will increase your confidence and reduce anxiety.
- Tools used to acquire knowledge
- Smartphone or tablet can easily access knowledge anywhere.
- Another educational tool is an e-book reader, e.g. Kindle.
- A lot of material can be downloaded for your consumption.

- Learn from free caregiving videos that are available on YouTube.
- Attend meetings, workshops, and online courses.

Benefits of Being Knowledgeable in Caregiving

- Knowledge is powerful.
- It increases confidence and self-esteem.
- Reduces anxiety and fear of the unknown.

Why Caregivers Learn Basic Communication Skills from Health Professionals

- Basic communication is essential to every relationship, especially in a caregiving situation. It does not matter what you say, but how you say it determines how the message is received by the sick person and the family.
- Excellent verbal and written communication skills are valuable in the caregiving industry.
- The caregiver must have the ability to listen, effectively interpret and correctly express him/herself confidently to facilitate conversation between the patient, family and caregiving team.
- Be polite, considerate, generous, affectionate, open, forgiving, understanding, and loving.
- Caregivers must go out of their way to make others feel special and confident in themselves.
- Be sensitive to other people's feelings.

- Be concerned about what others say.
- Listen to what others are saying.
- Voluntarily serve others without expecting anything in return.

Qualities of an Effective Caregiver are:

Empathy:

- Understanding what the person you are caring for is going through – their problems, challenges and desires. This improves relationships and the care you provide.

Accepting or Reaching Out:

- Look for help if you need assistance.
- This help can come from family members, health professionals, friends, neighbours and anyone who volunteers.

Compassion:

- The caregiver's ability to understand other people's challenges.
- Ability to tune into other people's distress and understand how to alleviate it.

- This attribute is important because most home care clients are in distress or in painful situations like post-operative, memory loss, etc.

Keen Observation Skills:

- Ability to spot when something is not right and report to relevant health personnel.
- The caregiver needs to have the ability to understand other people's challenges and have compassion for their situation.
- Talking and listening may not be enough as the patient may be trying very hard to hide something from you.
- On home visits, keep a sharp eye for any changes in the patient's condition. Also be aware of the client's environment.
- Notice and report any changes in condition and report to health personnel.

Interpersonal Skills:

- A caregiver always needs to interact with people as well as have a high level of social skills.
- The caregiver must have social skills to establish rapport, build trust and nurture a strong relationship with clients.
- This is good for both the client and caregiver and dispels isolation and loneliness.

Time Management:

- Make sure things are done within the specified time.
- There is therefore a need to prioritise tasks.
- Work efficiently to avoid being bogged down which is time-consuming.
- Have schedules and timetables so the patient and all members of the care team are satisfied.

Organisation:

- Know where everything is, e.g. where to find medication in the care recipient's home.
- Have things in their right places so you can find them in an emergency.
- Patients appreciate when things are done orderly and there is a structure to follow.

Cleanliness:

- The caregiver should only do light housekeeping so as not to be overloaded with work that is not their responsibility.
- Keep the client's environment very clean.

Patience:

- Clients tend to be irrational and irritable due to their health condition.
- Caregivers need to be calm and collected at all times.
- Do not rush as a caregiver. Speak slowly using simple language so you are understood by all and there is no misunderstanding.

Flexibility:

- The mindset of the caregiver must be flexible so as to handle unpredictable situations with grace and dignity.
- The caregiver needs to change the care plan if and when the client's condition dictates.
- Reschedule events to accommodate the care recipient's needs.

Initiative and Proactive:

- Caregivers usually work independently in the home with very little supervision.
- They are capable of making appropriate informed decisions when the patient's condition changes.
- They also know what action to take in an emergency.

Technical Skills:

Caregivers need to be proficient in using medical equipment like the following:

- Blood pressure machine.
- Heart rate monitor.
- First aid tools, mobility aids (wheelchairs, crutches, canes, walkers,
- motorised beds, orthotic or prosthetic devices).
- Digital devices to properly do their work.

Positivity:

- Caregivers need to approach caregiving with confidence, positivity, a happy attitude, and helpful approach.
- Everyone around should be influenced to provide quality care as a result of the caregiver's positive outlook.

Reliability:

- The care receiver depends, relies and trusts on the consistency of the care you provide.
- Things like providing transport, administration of medication, and companionship must be predictable.
- Lack of reliability would affect health, state of mind and mood.

Time for Self:

- Find time to care for yourself and your hobbies.
- Spend time at day-care services to reduce stress.
- Keep in contact with peers to socialise and recharge.

Mutual Respect:

- Listening and acknowledging feelings of the care recipient makes everyone feel secure.
- The recipient and caregivers are pleased by kind words.
- Patients like to be respected and not to be treated like invalids. Don't talk down to them.
- Caregivers are demoralised when their dignity is stripped or not observed.

Objectivity:

- Give the care recipient a chance to do things independently such as:
- Washing their glasses, making the bed, replacing a dish towel.
- This makes the care recipient feel as though they are accomplishing something without help.
- The caregiver's aim should be objectivity about the care recipient's opinions and wishes about caregiving.
- Check things not done on the to-do list.

Limitations:

- Know when to say you cannot go beyond a certain time, e.g. sleep at specific times, have me time without disturbances and have regular breaks.
- Reach out for help, nurture yourself. When your health fails, you are not useful to the care recipient.

Balance and Boundaries:

- The caregiver should balance his physical, emotional, mental, spiritual and social needs without feeling guilty of neglecting the care receiver.
- Say no when appropriate, otherwise your relationship will be affected by resentment and guilt.
- Where there are no boundaries the caregiver moves blindly and falls into serious emotional burnout.

There Must be a Limit as to What the Caregiver Does for Themselves and for the Care Recipient

- The caregiver must learn to say no to demands.
- Set realistic goals.
- Communicate firmly as to what should be done in a day.
- Have times for calls and visits.

Planning Skills:

- Set realistic goals for what you want to accomplish.
- Make a list of what needs to be done.
- Identify friends, family, outsiders, neighbours, agencies.
- Make schedules to cover a 24-hour shift if necessary.
- Give yourself respite periods to prevent burnout.
- Set limits and stick to them; learn to say no to loved ones.
- Form partnerships with the healthcare team because the caregiver has an important role in the care of the care receiver.
- Get involved in both short and long-term planning.

Problem-solving Skills

Caregiving involves problem-solving. Below is the process of doing it:

- Identify and discuss problems that exist now and could be in the future.
 - Be clear about what the problem is.
 - Remember, different people have different opinions.
- Understand the individual's interests.
- Identify possible solutions and act.
- Evaluate potential solutions and results.
- If the condition worsens, create short and long-term strategies.

6

HOW TO MAKE A HOME CAREGIVING PLAN

What is a Home Caregiving Plan?

It is a schedule of routine services that are organised at home to address some of the care recipient's pressing needs. These can include feeding, dressing, toileting, medication management, shopping and transportation.

What Does a Care Plan Entail?

- It describes the needs and problems of the patient.
- Their views, concerns, wants, preferences, views and choices.
- The available resources, goals and actions of the care team.
- Who will provide services to solve problems.
- Who will decide on frequency of services.
- Care management arrangements.
- Who will decide on frequency of assessments.

The Problem-Solving Process is:

- The act of defining, identifying and discussing the problems that exists currently and what could exist in the future.
- Determining the cause.
- Prioritising and selecting alternatives for solutions.
- Determining long and short-term strategies.

How to Determine Caregiving Needs

- Assess the current caregiving situation.
- The caregiver collects data – medical history, personal history, legal and financial documents – from the care receiver and addresses any problems at hand.
- Review the home environment, everyday tasks and health status of the client.

Assess All Areas of Everyday Tasks Such as:

- Personal care – find out if the care recipient can care for self, such as bathing, dressing, shaving, brushing teeth, washing hair, eating issues, using the toilet and using the phone without assistance.
- House care – determine if the patient is capable of cooking, booking doctor's appointment, etc.

- Medication management – assess if the patient has the ability to look for physical therapy or have a mediator to resolve family conflicts.
- Emotional care – determine the care receiver’s ability to have companionship and meaningful conversation.
- Home safety – assess if there are any hazards in the home.
- Does the yard need maintenance?
- Find out if there are any stairs for the physically challenged and how they can get assistance when alone.
- Finances – what is the expected expenditure and source of income for caregiving.
- Legal issues – does the patient have a signed will or valid health insurance?
- Does the primary caregiver have access to the above?
- Lifestyle – determine hobbies for the care recipient. Are there friends who visit? Look at church attendance. Try to ensure the care receiver is not isolated.

Arrange for a Detailed Medical Assessment and Diagnosis

- Make an appointment with the doctor.
- Take patient to doctor for thorough medical assessment.
- Introduce yourself as the primary caregiver and ask for a briefing about disease progress.
- Involve the patient at all times.
- Learn more about the disease and relevant treatment through research.
- Talk to health professionals as care recipient’s advocate.

- Ask necessary questions about the patient’s condition for a definite diagnosis and treatment.
- Discuss with the care recipient your observation and options to solve problems.

Identify Needs and Set Goals

- Ask what is lacking in the client’s routine?
- What are the objectives to be achieved?
- Based on the above results, the assessment is complete.
- Make a list of shortcomings, concerns, preferences and wants in order of priority.

Form Caregiving Team

A Caregiving Team Should Include:

- Care recipient, all immediate family members, and significant others in the initial discussion.
- Close friends, neighbours and volunteers, community support (church members, clubs, organisations).
- Professionals from home care, accountants, lawyers and paid caregivers.
- Social services, agencies, e.g. adult day-care centres.
- Have contact information of all care team members.

Examine the Caregiving Team for the Following:

- Expertise, abilities, interest, skills, responsibilities, strengths and weaknesses.
- Group them according to those who are hands-on and are comfortable giving direct care and helping the patient.
- There are those who are hands-off and prefer doing tasks like finances.
- Others can do errands like doctor's appointments, household chores, etc.
- Make a list of viable solutions for each problem in the care plan, e.g. client losing weight, so a family member volunteers to bring meals at specified times.
- Make schedules and timetables for all to follow.
- Distribute plan, choose a person to follow up and co-ordinate caregiving tasks and report to the group.
- Create a communication platform online for the group so all know what is happening.

Identify Resources Such as:

- Hiring a care manager, social worker or geriatric care manager to guide on resources available in the community, private organisations, area agencies, and government institutions.

- Professional help guide, where the best care is, advice on community resources.
- Assisting on arranging for services and continuity of care.

Implement Plan**The Care Team Works Together to Develop a Comprehensive Strategy to Provide Care Such as:**

- Writing the care plan.
- Executing plan as written.
- Ensuring everyone has a copy of the plan.
- Checking on care team members regularly to ensure they are on target.
- Care team should assign a co-ordinator for each area of care.
- Determining roles of team members.
- Be honest with yourself.
- Find the best way to communicate such as Skype, WhatsApp and online.

How to Interpret and Evaluate Results?

- Review as needs change and roles are clarified, as and when necessary.

- Display open communication, clear roles; show appreciation to make the caregiving team motivated to achieve results.
- Results may indicate the need for:
 - New medical or hospice care.
 - Change of living accommodation, or increase in activity.
 - Identifying where gaps are and prioritising what needs to be changed.
 - Looking into finances to see if they need supplementing by family or care recipient will manage.

Emergency Readiness in Caregiving

Caregiving will always have unexpected surprises such as:

- Accidents when a patient falls and hospitalisation is needed.
- Fire, disaster, power failure, appliance failure.
- Illness of worker or leaves without notice.
- Financial difficulties, e.g. funds run out. Expected money not received.
- Natural disasters, e.g. floods, earthquakes.
- Housing becomes unsafe to live in.
- Care provider crises, e.g. becomes ill and does not turn up for work.
- Consider potential problems and those that exist, be prepared to take action and make necessary changes.

- Keep phone numbers of everyone who matters.
- Determine if a person responsible for legal decisions has been identified.

What are the Plans for Handling Caregiving Emergencies

- Document and communicate how the team will be notified.
- Who steps in to care if you are not there.
- What backup plan is in place for anticipated emergencies.
- Communicate the plan to care team members.
- Update emergency plan regularly.
- Keep all participants updated about the situation.
- Use current, efficient and cost-effective methods of communication, e.g. Facebook, Twitter, Skype and Zoom.

7

INFECTION CONTROL

Family Caregiver Role in Infection Control, Risks and Prevention

Worldwide, countries have developed specific guidelines to prevent and control the spread of infectious diseases.

The Botswana Government Gazette Extraordinary Supplement C (2022) March 18 stipulates that control measures must be followed to prevent the spreading of diseases.

General Information

- Infection control is one of the most serious issues everyone should learn about – especially in home caregiving.
- Caregivers usually adopt the caregiving role without any experience in caregiving issues.
- Lack of training in home caregiving can cause concern.

Importance of Infection Control

- The reduction of hospitalisations can save lives.
- Home care clients already have compromised immunities.
- The elderly are more prone to infection when they are ill.

Types of Potential Infections

- Skin infections, urinary tract infections, influenza and pneumonia are the main concerns among the elderly. Sepsis is worse when it develops in addition to other infections.
- Sepsis is described as an overreaction of the body to an infection. It develops quickly and is life-threatening, causing extensive damage.
- Usually, the elderly do not survive sepsis.

Symptoms of Possible Infection

- In pneumonia and skin infections, symptoms are common and easily identifiable.
- Symptoms include agitation, hallucinations, confusion, rapid breathing and shaking.
- Temperature of 37.8°C or more.

Method of Prevention

- Wash hands thoroughly at all times.
- Anyone coming into contact with someone with a cold or any communicable disease must wear a mask and gloves when caring for the elderly.
- Bedding to be washed frequently.
- Sterilise medical supplies like thermometers.
- Cover all wounds to prevent infection.
- Those with diabetes should be inspected for wounds and the wounds dressed as they are prone to infection.

Vaccinations Against Infectious Diseases can Reduce the Risk Of Illness

- Insist on pneumonia, flu, and shingles vaccinations for anyone prone to these illnesses.
- Antiviral drugs can help to reduce severity of the flu and assist with pneumonia recovery.

Infection Control

Home Care for Covid-19 Positive People and Those not Infected

With the emergence of infectious diseases like Covid-19 and the inability of health facilities to cope with the number of patients, home care is the next best alternative for care. There

is a need for patients and family caregivers to learn how to provide safe, quality care at home and not spread infections to the entire community.

It is advisable for the entire family to plan how Covid-19 can be prevented from spreading within the family and to the community.

Below is a strategy to contain the disease while providing care and support to new caregivers.

Create a Caregiving Team and Consider the Following:

- Choose healthy family members who are not high risk, e.g. not suffering from high blood pressure, diabetes, cancer, etc.
- Include friends and neighbours who will help with key caregiving tasks.
- Identify community services such as meals on wheels and other services like medicines to help relieve the load.

Take an Inventory of Essential Items Such as:

- Determining what and how much food will be needed.
- Medications and health supplies for the care recipient.
- Stock water and cleaning materials.
- Medical supplies and equipment.

- Medicine to reduce temperature.
- Reschedule appointments if necessary or use virtual visits.
- Over the counter medications, e.g. paracetamol for pain and cough mixtures.

Make a Plan to Stay Connected by:

- Isolating what is crucial.
- Social distancing by one to two metres.
- Use of technology to facilitate group communications, i.e. team phone calls, Skype, Google, Facebook, Zoom, texts and online communication.
- Have reading materials using the safest methods.

Observe Personal Safety and Self-Care

- Stay healthy and limit social contact, e.g. visitors.
- Take care of yourself.
- Plan for replacement in case you get ill.

Emergency Preparedness Plan

- Have displayed an emergency list with contact details.
- Create a list of emergency supplies such as food, toilet-ries and water.

Have a Communication Plan

- This is crucial in isolation and social distancing situations.
- Use technology for Team phone calls, Google, Skype, Facebook, Zoom, text and online communication.
- Arrange for safe delivery of reading materials.

How to Diagnose a Positive Covid -19 Patient

Signs and Common Symptoms of Covid-19:

- Fever of 37.8°C or more
- Oxygen rate: 95%
- Dry cough
- Respiration: 25
- Heart rate: 120 beats per minute
- Normal mental status

Less Common Symptoms:

- Fever or chills, cough, shortness of breath
- Muscle or body aches, pains and fatigue
- Loss of smell and taste
- Loss of appetite
- Headache, nose congestion
- Running nose

- Nausea and vomiting
- Diarrhoea

**Seek Emergency Medical Attention if You
Have the Following:**

- Trouble breathing
- Continuous chest pain and heavy pressure
- New confusion
- Pale, bluish colour on lips and nail beds
- Finally, test for Covid-19

**To Prepare a Room for Isolating a Covid-19
Positive Patient, do the Following:**

- Preferably the room must contain a toilet. If not, the seats and door knobs must be disinfected before and after use.
- Provide everything the sick person will need for 14 days, e.g. soap, toothpaste, toothbrush, body lotion, deodorant.
- Have a box of masks and gloves, as well as hand sanitiser.
- Put extra sheets, blankets, towels and sleepwear in the room.
- Have a small table with stationery, pencils, television, computer and reading material.
- Supply adequate water.

- Have a waste bin lined with plastic to dispose of all waste generated by the sick person.
- Do not share items in the isolation room with the rest of the family.
- Room to be well ventilated.
- Disinfect room and non-disposal items after the sick person is free from illness – after about 10 to 14 days.

Equipment to Use at Home

Gown, thermometer, alcohol wipes, eye protector, disinfectant, pain killers and paper towels.

How to Care for a Patient at Home

- Provide support.
- Protect yourself.
- Limit contact with the sick person.
- Assist the sick person with basic needs.
- Provide plenty of water to drink.
- Watch for warning signs.

How Caregiver Reduces the Risk of Covid-19 Infection

- Be vaccinated as soon as eligible.

- Continue to observe Covid-19 protocols until vaccinated.
- Wear a mask over the nose and mouth.
- Keep a distance of one to two metres from people.
- Avoid crowded areas and poorly ventilated rooms.
- Wash hands with soap and water – often.
- Sanitiser should be 60% alcohol.

**To Protect Yourself from Covid-19, Wash your
Hands at the Following Times:**

- Before preparing meals.
- Before eating.
- After leaving the client.

8

HOME CARE BASIC PROCEDURES

Botswana Qualification Authority Manual (2021) provides and maintains a national qualification framework to co-ordinate the quality assurance system of education, training and skills development. Therefore, the basic core content of the home caregiver curriculum will be determined by the regulations of various countries.

**Below is a Home Care Guide to Provide
Personal Hygiene Care**

General Tips

- Caregiver assists with everyday tasks as needed by the care receiver.
- Caregiver establishes short-term goals.
- Encourage participation, independence, motivate self-care and educate the use of assistive devices.
- Establish a routine schedule for daily grooming.

- Have activities at the same place and same time, e.g. eating breakfast, brushing teeth, bathing mornings and evenings.
- Choose the most convenient time with no disturbances for bathing, grooming and other activities.
- Caregiver discusses with care recipient when to assist with everyday tasks such as bathing, getting in and out of shower or bath, shampooing, shaving, nail cutting, haircuts and other grooming needs.
- Caregiver ensures the safety and that things are done properly.

Procedure: Bed Bath

What is a bed bath? This is a bath done to wash someone who cannot get out of bed. You may need to wash the entire body or just help wash certain areas.

Supplies Needed to Give the Bath

- Have the following within reach:
- Two water basins.
- Two bath towels.
- Two wash clothes.
- Two pairs of gloves.
- Soap, lotion, deodorants.
- Clean clothes to change into.

How to Prepare the Room

- Keep the room warm.
- Close windows.
- Ensure there is privacy and no intruders. Doors should be closed and there is no draught in the room.
- Consult home care nurses if you are not sure how to bath a sick person.

Initiate the Activity

- Ask for bathing preference.
- Adjust water temperature to what is comfortable for the sick person, or use your elbow to test the water.
- Remove top bedlinen and cover the patient's body with bath blanket or large towel.
- Remove patient's clothes while keeping body covered with bath blanket.

How to Give a Bed Bath

- Fill the two basins with warm water; one for bathing and the other for rinsing.
- Place a big towel under the person to protect the bedding from getting wet.
- Ensure you prevent the person from falling out of bed if you walk away for any reason.
- Wet the washcloth without soap.

- Gently wipe one eyelid from the inner corner. Repeat for the other eyelid. Pat eyelids dry.
- Wash face, neck and ears.
- Place the towel under the head and neck while gently washing the face, neck and ears with a clean washcloth – not dripping wet (soap dries the face, so avoid using soap).
- Rinse the face cloth, and pat dry the parts you have just washed.
- Wash the farthest arm, shoulder and underarm with soapy water, supporting the elbow as you wash the arm. Rinse and pat dry.
- Wash the arm nearest to you. Rinse and pat dry as before.
- Expose and wash the chest and abdomen with soapy water. Cover the rest of the body with a towel.
- For females, wash under each breast and pat dry.
- To protect the bedding, move the dry towel and place it under the far leg and knee. Wash, rinse and pat leg dry.
- Put foot in a basin of water, wash foot and between toes.
- Wash the leg nearest to you.
- Turn the client towards you and wash the back from the shoulder to the buttocks.
- Apply lotion as desired. Treat pressure areas.
- Discard gloves.

How to Care for the Perineal Area

- Change water in both basins and wear a pair of gloves.
- You may bend the person's knees to reach the genital area.
- For women, wash genitals from front to back to prevent infection.
- For men, wash around the testicles and between the buttocks.
- And may need to help the person roll on to the side.
- Leave the client in a comfortable position.

Last Step after a Bed Bath

Treat bed sores – how to speed up the healing process.

Small Areas

- Clean gently with salty water.

Moderately Deep Sores

- Expose the wound to air and give paracetamol for pain as needed.

Deep and Extensive Areas

- Clean gently with salty water and cover with clean light dressing to encourage healing.
- Rub lotion on to the arms, legs, feet and all dry areas.
- Discard what needs to be thrown away, clean and tidy up.

Caring for a Care Receiver who is Partially Dependent

(Caregiver to seek professional training in lifting, transferring and moving of patients)

- Limit full tub or shower wash to once or twice a week.
- Use a bath chair for sitting in the shower, bathtub or toilet seat to sponge-bath the sick person.
- Install bars on bath and chair for safety.
- Avoid oils in the bath or shower to prevent slipping.
- Use a handheld shower if the person is unable to bathe himself.
- Place your hand over the sick person's hand and assist with washing all over the body.
- Ensure areas such as genitals, armpits, skin folds, and any difficult to reach areas are washed and dried after a shower or bath.
- Get the patient out of bed to a chair as many times as possible during the day.

- When changing position, do not drag the patient but lift to avoid breaking the skin.
- While in bed, encourage the patient to exercise often.
- Patient to change position every two hours to avoid pressure sores.
- Use pillows to support the desired position.
- Watch for damaged skin – such as colour change – on the back, hips and shoulders where there is always pressure.
- Something soft under the patient would be ideal.
- In case of leakage, protect the skin around private parts and rectal area to prevent irritation from urine and stools.
- Gently moisturise pressure points – back, hips, elbows, ankles.
- Bedlinen to be washed, aired and dried frequently outside the house.

Procedure: Grooming

- Let sick person comb hair if he can or go to a hair salon if condition allows.
- Use a large comb if available.
- For washing hair, try using a sink if a bath is preferred to a shower.
- Use a hair shampooing tray.
- A dry shampoo could also be used for persons who do not like to wash hair.

Hair and Shaving

- An electric razor is better for patients with a tendency to bleeding as this avoids cuts.

Fingernails

- Cut fingernails without injuring the patient.
- Keep them trimmed and clean.

Oral Care

- Teeth and dentures to be cleaned daily. If in doubt, consult a dentist for good dental care advice.
- If the sick person is unable to brush their teeth, assist with putting toothpaste on the toothbrush, then help them open their mouth and brush teeth only on the outside of the teeth.
- Clean dentures daily. Make sure they fit properly and examine gums for sores and redness.
- The mouth is to be rinsed after meals.

Foot Care

- Soak feet in warm soapy water if dry and flaky.
- Cut toenails.
- Keep them trimmed and clean.

Procedure: Dressing

- If the person is able to dress himself, then lay out clothes and let him do as much as he can.
- Assist person when it is difficult to open zippers and buttons and bend and stretch arms.
- Avoid pullover shirts or clothes with zippers at the back.
- Clothes that need bending and arm movement are difficult for senior citizens.
- Buy clothes that are easy to put on and take off, to zip and unbutton.
- You can shop from special shops for home-care clothes, shirts, dresses and gowns. Use catalogues.
- Garments and shoes should be easy to close. The same applies to belts, buckles, buttons and shoelaces.
- Use a grabber to reach socks.

Procedure: Eating/Feeding

- Provide a balanced diet with plenty of fluids.
- When preparing meals, plan various aspects of meals including shopping and storing groceries.
- Depending on the care recipient's condition, serve finger foods that are easier to eat.
- When eating and to avoid choking, get the patient to sit up if able.
- Ask for a sick person's preferences for time to eat and what to eat.
- Serve small portions and help to cut food for a sick person.
- Make use of assistive devices such as plate guards or silverware that has two special handles for persons who have difficulty handling utensils.
- Use large utensils if the hands are weak and shaky.
- Buy special plastic spoons, forks and knives.
- Adults should not be forced to eat.

Managing Symptoms – Feeding and Weight Loss

- Encourage the patient to eat, but do not force them to eat.
- Offer small amounts as appreciated by the patient.
- Let the patient have a variety of foods to choose from.
- Seek medical help if the condition deteriorates.

Procedure: Mobility Assistance

- When moving in and out of bed, encourage the sick person to sit and dangle their legs before standing. This allows the body to adjust to the change of position and prevents dizziness and falls when blood flow is altered.
- Seniors have difficulty moving around because of painful joints and need assistance by a caregiver, which helps as exercise for seniors and means they remain safe in the house.

Procedure: Toileting and Managing Incontinence

- This act is performed by a caregiver for seniors several times a day.
- Modify the bathroom and toilet if necessary.
- Install safety features in the bathroom such as grab bars and a raised toilet seat.
- Replace an ordinary toilet with one for the disabled.
- Have a bedside commode or urinal if getting up to the bathroom is a problem, especially at night.
- Schedule routine bathroom visits to prevent accidents.
- Preserve dignity at all times and use assistive devices where necessary.

- Inform the doctor about a loss of bladder and bowel control. It may be a symptom of a bigger problem and medication could be the solution.
- Ensure ample supply of disposable sanitary wear – briefs, pads, disposable sheets, disposable gloves.

Care of Rectal Area

- Clean rectal area with toilet paper.
- Wash the rectal area with soap and water when necessary.
- Apply barrier cream to the rectal area.
- Consult the doctor if there is broken skin around the rectal area.

Changing Diapers for Bedridden Adult Patients

- Supplies:
 - Two pairs of gloves.
 - Clean diapers.
 - Plastic bag.
 - Wet wipes.
 - Barrier cream.

- Wash hands and wear gloves.
- Raise the bed, if adjustable, to a comfortable position.
- Patient to lie on his/her back.
- Head to be horizontal.
- Unfasten the tabs on the soiled diaper, tucking the inside farthest away from you under the hips.
- Placing one hand on hip and the other on the shoulder, roll the patient away from you on to their side.
- When the diaper is tucked far enough under the hip, you should be able to pull out the diaper from under the patient.
- Roll the soiled diaper inward as you remove it to contain any mess.
- Place soiled diaper in a plastic bag, but do not close it yet.
- Using moistened wet wipes, clean the diaper area front to back.
- Be gentle not to break the skin.
- You may need to roll the patient on to their back and clean any areas you cannot reach.
- Dispose of wet wipes in a plastic bag.

Check Pressure Sores to Treat them Early

- Clean and apply barrier ointment.
- When skin is dry, tuck the diaper side farthest from you under the hip, flatten and position the diaper on the bed.

- Pull the diaper between the patient's legs; close the diaper with tabs facing up and remove wrinkles.
- Discard the plastic bag.
- Wash hands.
- Turn patient every two hours.

Providing a Bedpan

- Sprinkle powder on the bedpan rim to prevent sliding.
- You can use a bed pad under the bedpan to prevent spills.
- Help get the bedpan into position.
- Help the person sit up or lift hips slightly, or slide the bedpan under the person.
- If there is a helper, let him/her slide the bedpan under the person's buttocks while you help lift their hips.
- Alternatively, choose to turn the person on to their side.
- Press the bedpan gently against the person's buttock and help him/her turn back on to the bedpan.
- If safe, leave the person alone to use the bedpan in privacy.

After Bedpan Use

- Hold the bedpan in place and help the person roll off it, away from you.
- Carefully set aside the bedpan.

- Help the person wipe himself/herself if need be.
- Ensure the bedpan contents are not spilt over the sick person and bed linen.
- Empty the bedpan into the toilet.
- Rinse and wash the bedpan using disinfectant diluted with water.
- Dry the bedpan or let it air dry.
- Discard gloves.

Manage Medication

- Collect prescriptions.
- Supervise that drugs are taken as prescribed.
- Monitor side effects.

Manage Household

- Assist with cleaning.
- Tidy up, declutter.
- Do laundry and pack linen into the wardrobes.

Managerial Tasks

- Manage finances, banking and paying bills.
- Apply for financial assistance and social services.
- Take care of legal affairs.

- Look after medical appointments.
- Look after pets, children and visitors.

Transportation and Shopping

- Organise transport to take care receiver to doctor's appointments.
- Procure supplies, groceries and medications.
- Drive and assist to recreational facilities.

Shopping is a Time for:

- Socialising.
- Meeting people.
- Getting help with shopping.
- Using meals on wheels.

Companionship and Mental Support

- Help sick the person have a positive frame of mind.
- Organise visits from different people.

Communication

- Assists in managing household phones and mail.
- Makes the home hospitable for visitors.

Companionship and Conversation for the Care Receiver

- Seniors are prone to loneliness and need companions with whom to play games, watch TV, and go for walks.

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COMMON PROBLEMS AND SOLUTIONS

Headache:

- Give paracetamol for adults if the pain is mild.
- Report to the doctor if pain persists.

Nausea and Vomiting:

- Food to be determined by the patient's tastes, likes and illness.
- Offer frequent dry meals, not oily.
- Seek medical help if the condition worsens.

Painful Mouth Ulcers or Pain in Swallowing:

- Avoid extreme temperatures of food – hot, cold, spicy.
- Use a soft toothbrush or soft cloth to remove mouth debris.
- Provide soft food such as potatoes, milk, honey or porridge.
- Refer to doctor if the patient gets worse; has persistent sores; smelly mouth and difficulty swallowing.

Dry Mouth:

- Give frequent sips of water; moisten lips with water and lip balm.
- Seek the doctor's advice if not getting better.

Coughing and Difficulty Breathing:

Local remedies

- For a simple cough, use honey and lemon to soothe it.
- Steam with menthol and eucalyptus to ease breathing.
- Help patient sit up.
- Use extra pillows.
- Open windows to allow air in.

- Give patient water, and hit patient's back and chest to loosen sputum and make it easier to cough.
- Refer to the doctor if there is blood in the sputum or not better.

How to Handle Sputum

- Protect yourself as sputum can be highly infectious. Use gloves.
- Use a small container with ash and sand on the bottom for sputum.
- Cover and dispose of safely.

Treatment of Hiccups:

- As fast as possible, drink cold water, eat two heaped teaspoons of sugar and drink water.
- Breathe into a paper bag and stop when uncomfortable.
- Hold your breath and stop when uncomfortable.
- Pull knees to the chest and lean forward to compress the chest.

Help with Worries:

- Listen attentively to the patient.
- Confidentially discuss the problem.

- Soft music or massaging relaxes the muscles.
- Meditation makes one be at peace, refer for professional counselling.

Sleeping Problems:

- Pay attention to fears that make the patient stay awake.
- Adopt a quiet atmosphere at night.
- Avoid stimulants like tea and coffee.
- Attend to pain if present.
- Give a soothing drink at night, e.g. warm milk, almond milk or decaffeinated green tea.

Care for Confused Patients

Signs and Symptoms

- Forgetfulness, lack of concentration, trouble speaking.
- Mood swings, insults and unacceptable behaviour like undressing in public.

Solutions

- Patients to be kept in a constant environment where things are predictable.
- Things for daily use by the patient must be within easy reach.

- Routine daily activities must be at a regular place and time.
- Dangerous objects must be removed.
- Communication must be very clear.
- The place must be quiet.
- A change in setting or caregiver can cause increased confusion in patients. Avoid change as much as possible.
- Medications to be taken as prescribed.
- Patients to be given quality care at all times.
- Caregiver to be respectful of the patient at all times.
- Any concerns about the patient's condition to be referred to the doctor.

Diarrhoea:

- Increase fluids such as water, rice and soup.
- Make oral rehydration solution (one litre of water plus one cup of salt) and give frequently.
- See the doctor if the patient has a fever, is vomiting, has blood in stools or diarrhoea for five days.

Difficulty in Passing Stools (Constipation)

- Examine for rectal impaction (use gloves).
- Encourage exercise if possible.
- Regular toilet routine.
- Add roughage to meals – fruit and vegetables.

- Use local herbal treatment such as dried pawpaw seeds mixed with half a teaspoon of water.
- One teaspoon of vegetable oil before breakfast.
- Give a small soap enema (into the rectum).
- Seek medical assistance if there is no improvement.

Incontinence:

- Control stains and odours
- Include cranberry juice in the diet to reduce urine odour.
- Protect the mattress with a rubber/plastic cover and sheets.
- Consider a breathable, washable layer like sheepskin between sheets and waterproof material to avoid access sweating.
- Remove soiled bedlinen and clothes quickly and wash immediately.
- Rinse stained items and soak in dishwashing detergent to loosen stains.
- Clean bedpans, urinals and commodes with household cleaners.
- Avoid odours on furniture and household items by cleaning soiled areas with water mildly diluted with vinegar.
- Protect furniture with disposable or waterproof pads.

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PALLIATIVE CARE

What is Palliative Care?

- Palliative care is care given at home to improve the quality of life when someone is terminally ill.
- Palliative caregivers are usually overwhelmed by emotions and responsibilities that go hand-in-hand with the end-of-life stages.
- The family supports the ill person and makes appropriate decisions regarding care.

How to Prepare the Patient and Family for Death.

When providing emotional healthcare near end-of-life, observe the following:

- The patient expresses fear, anger, sadness and acceptance.

- The caregiver must listen and show understanding about what the patient is going through.
- Address patient's concerns like custody of children and their upkeep and funeral costs.
- Address spiritual aspects of the patient while respecting his feelings.
- Be compassionate and willing to talk about the patient's concerns.
- In conjunction with the healthcare provider, stop some medicine or drugs.
- Control symptoms to relieve discomfort such as diarrhoea.
- Continue treatment where the disease is infectious – moisten lips, mouth and eyes.
- Keep patient clean.
- Turn patient every two hours.
- Call religious leaders as requested by the patient.

The Role of the Palliative Team is as Follows:

- Collect the history of the sick person to determine the problem.
- Conduct a comprehensive assessment of the client and their family.
- Co-ordinate care provided to the care receiver.
- Supervise giving the client's medication.
- Consist of family, friends and health professionals.

- The primary caregiver is the best person to talk to health professionals on behalf of the sick person.
- The preferences, needs and goals of the children should be communicated by the caregiver.

Palliative Care for Children

How to Discuss Death with Children

- Ask children to say what they know about death.
- Or take them to a sick individual and explain how the illness deteriorates until the person dies.
- Explain how death is a normal stage in life and we all go through it.
- Be age-appropriate in language and examples.
- Being honest – very important as the child asks more questions.

Caring for Children of Dying Parent

- Talk about the loss of the parent.
- Do not avoid talking to them.
- Talk about the parent's illness.
- Make children feel loved.
- Meditate, sit and think about your carefree life.

Providing End-Of-Life Care and Adapting your Home

- Decide where care will be provided – sometimes, a hospital might be chosen.
- A room upstairs, type of toilet and possibly a hospital bed might have to be chosen for caregiving.

Other Considerations

- The floor and hallways must be clear of rugs as they are a potential hazard.
- For the patient to have a view of the outside, place the bed near a window.
- The patient's room must be decorated beautifully with their favourite items.
- Provide a bedside table within easy reach to keep items needed by the patient.
- Place an armchair near the bed. Have a monitor or bell nearby for calling for help.

Equipment for End-Of-Life Care Includes:

- Hospital bed.
- Bedside lamp.
- Bedside commode.
- Raised toilet seat.
- Walker.
- Wheelchair.

- Bath seat.
- Disposable briefs, pads, undergarment protective clothing and bedding.

The above can be rented or hired to provide end-of-life care.

Caring for a Dying Child

- Prepare to talk and answer questions.
- Do not avoid.
- Answer questions in simple terms.
- Ensure the child has company.
- If and when possible, family members should play with the child.

After-Loss Grieving

- It is natural for anyone to mourn, which may last for months or years.
- There may be religious rituals/events or time to be with friends and family to share feelings.
- Feel free to express loss and sadness at any time.

Physical Symptoms of Grief

- Lack of appetite.
- The stomach is painful and upset.
- Loss of sleep.
- Worrying, tiredness, less energy.
- Depression, suicidal thoughts.

Living with Grief after a Loved One's Death

How to cope.

- Look for people who care – like relatives and support groups as they understand what you are feeling.
- Express feelings by talking to others.
- Do not be dependent on drugs and alcohol.
- Aim to accept what has happened as a part of life.

11

THE STRAIN EXPERIENCED BY A FAMILY CAREGIVER

Strain is experienced when a caregiver feels overwhelmed and fails to perform to the best of their ability. This is followed by a feeling of stress and anxiety. Many caregivers experience caregiver role strain.

What can Cause Role Strain?

- When a caregiver does more for the care receiver and ignores their health.
- An increase in expenditure on care recipient.
- Financial burden for caregiver.
- Sleep difficulties brought on by the care recipient not sleeping.
- Distress and irritation.

Signs and Symptoms of Role Stress

- Sleep problems.
- Irritability.
- Social withdrawal.
- Change in appetite.
- Tiredness.
- Crying easily.
- If not treated, the caregiver will become depressed and need to seek medical help.

Preventing Care Role Strain

- The caregiver must do all he can to stay in good health so as to look after the care recipient.
- Adequate sleep – eight hours is essential.
- A balanced diet is essential – plenty of vegetables, whole grains, and fruit.
- Exercise for at least 30 minutes a day – do things that excite you; laugh a lot.
- Do not isolate yourself; keep in touch with friends.
- Be in control of your negative emotions; do not lose your temper or get angry; be patient.

Treating Caregiver Strain

- Find resources in the community.
- Ask for help and accept it.

- Focus on what you are able to provide and set routines.
- Get organised.
- Look after yourself.

Managing Caregiver Stress

- Devise a timetable of what others can do and match tasks to persons.
- Take sick person for walks, grocery shopping, or help with meal preparation.
- Delegate duties so that you do only a few things.
- Have realistic goals – divide tasks into small bits that can be achieved in the time you have. Let others do the rest. Set priorities.
- Network and find resources in your area, e.g. arrange special training regarding loved one's disease, caregiving services, laundry, transportation, meal delivery, etc.

Join Support Groups for Caregivers

- These groups can provide problem-solving strategies where there are difficult situations.
- The group has experiences to share, thus developing meaningful relationships.

Set Personal Health Goals

- Set time for when to retire to bed.
- Set time for daily exercise.
- Eat a balanced diet regularly.
- See a doctor for any health problems that persist.

Respite Care

A caregiver needs breaks no matter how difficult the situation is.

Types of Respite Care Found in Communities are:

- **In home respite:** This provides home care aides and nursing services at a cost.
- **Adult care centres or programmes:** Offers care for either adults or children.
- **Short-term nursing homes:** These places take in clients for short periods while providing relief to the caregiver.

Caregivers Who are at Greater Risk of Stress

Display the Following:

- They often stay in the same home with the care recipient.
- Socialising with friends is limited.
- They are depressed.

- They have financial problems.
- They spend long hours giving care.
- They do not have adequate coping or problem-solving skills.

Emotional Side of Caregiving

- Many emotions – positive and negative – take place during the caregiving period.
- These emotions are to be acknowledged but not suppressed.
- The caregiver should rather seek help for coping strategies.

Emotions and Coping Solutions while Caregiving as Shown Below:

Ambivalence

- The caregiver is undecided whether to address the situation or not.
- The solution is to accept the situation of not being in the mood on certain days. This is normal but should be temporal and the caregiver moves on.

Anger and Frustration

- Because the care recipient is uncooperative, irrational and in a fighting mood, the caregiver loses their temper.

- The solution is to cool down, find a reasonable way to express what happened, and talk to someone different.

Fulfilment, Doubt and Anxiety

- Care recipient's diagnosis results in conflicting emotions.
- The care recipient appreciates all the wonderful work done by the caregiver. The caregiver greatly rejoices.
- As the condition worsens, doubts and anxiety return, and the caregiver searches for more solutions, including relying on prayer.

Guilt, Frustration and Fear

- Caregiver at times spends many hours caregiving and feels stressed and inadequate.
- The patient's condition shows no improvement.
- The future is bleak. The care recipient is very miserable.
- The caregiver has no time for self-care.
- The solution is to take a break to recharge and rejuvenate. Forgive yourself for selfish thoughts.
- Seek counselling.

Feeling Trapped and Resentful

- The care recipient's condition declines.
- The caregiver feels helpless, as though not doing enough for a loved one.

- Take breaks to refresh and recharge. More energy is required to give care.
- Seek counselling.

Emotional Wellness

Has to do with our thoughts, feelings, and behaviour. It allows you to clearly recognise and accept your feelings both positive and negative. This includes:

- Adapting to stress.
- Life changes.
- Difficult circumstances.

Why is Emotional Health and Wellness Important?

- How you feel affects your entire life, e.g. sadness and depression can affect your personal, professional and quality of life.
- Difficulty managing stress or expressing emotions can decrease general well-being and lead to health problems.
- When emotions are controlled, individuals develop strong interpersonal relationships that, in turn, positively influence all aspects of their lives.

Ways to Improve Emotional Wellness

- Be optimistic and focus on the positive aspects of life.
- Learn to accept emotions whether good, bad or ugly.
- Build and maintain strong relationships.
- Stay in the moment.
- Practice mindfulness.
- Smile and laugh as much as possible.
- Maintain a good balance in life.
- Get enough sleep.
- Seek professional support as needed.
- Manage stress through positive coping strategies.

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SUPPORT NEEDED TO OVERCOME CAREGIVING CHALLENGES

Support is often needed to overcome caregiving challenges and make the process more rewarding.

What are the Best Community Services

- Counselling services.
- Caregiver services.
- Respite programme cancer association.
- Diabetes association.
- Adult day-care.
- Hospices.
- Elder care.
- Nutrition programmes.

What Resources are there to Support Caregivers?

- Community nurse, occupational therapist and social worker who may help with the following:
- Assist by making recommendations to modify the caregiver's house to make it more suitable for caregiving.
- Assist in choosing equipment and assistive device and aids.
- Get assistant caregivers and respite care.
- Some hospitals offer classes that teach caregiving skills suitable for your situation.
- Ask your doctor for referrals or call a local agency for guidance.

Find Resources in your Community to Help Such as:

- Adult day-care centres or respite services to give the primary caregiver a break from caregiving responsibilities.
- Ask and accept help.
- Make a list of what volunteers can do, e.g. shopping and cooking meals.
- Be in a support group.

What Caregiving Services are Found in the Community?

- Meal delivery.
- Private home-care agency services.
- Non-medical home care services, e.g. housekeeping, meal preparation, companionship, transportation to doctor's appointments, modifications in home such as rails, ramps, fixtures in the bath, etc.
- Respite care to relieve the burden carried by the caregiver.
- Consider paid assistance if the family is not available to help.
- Groups for caregivers.

What are the Benefits of Caregiver Support Groups?

Caregiving is overwhelming, and all people struggle with daily caregiving challenges.

- Do not suffer alone. Join a support group to make your life easier, reduce stress and improve your health.
- Have a sense of empowerment and control. This will help improve your coping skills and reduce stress, anxiety and depression.
- The caregiver gets a clear understanding of what to expect in a caregiving situation.
- Practical information and advice on treatment options are made available.

- Caregiving ability improved, thus enhancing a better quality of life to the care recipient.
- The care recipient stays at home.

How to Identify Local Caregiver Support Groups?

- Ask at hospitals and community centres for local support groups.
- For specific conditions, check websites for information and meetings, e.g. Alzheimer's Association, National Stroke Association, Cancer Association, Diabetic Association.
- Elder care.

Support Systems

- The caregiver needs a team and cannot work alone. The entire village is needed to give a helping hand.
- Where people are not willing to help, search on the internet for caregiving organisations or hired help.

Caregiver Support Networks

- Expanding your caregiving networks involves clarifying your needs as a caregiver and determining the best resources to meet needs.

- Needs could be listed and organised into three groups, which are:

1. Emotional (Needs) may Include:

- Someone who will listen; not be judgemental and not give unsolicited advice.
- Respite care; someone who can share the caregiving, providing an opportunity for you to rest and rejuvenate your body.
- A safe environment, a supportive space to vent, e.g. a group of caregivers to share experiences and discuss as a form of caregiver support.
- This allows for time to be pampered by other caregivers who have been there before.

2. Technical Assistance (Needs) may Include:

- People with information, knowledge, resources on finance, and legal expertise.
- Health professionals who know the disease may help you understand and cope with the illness and suggest appropriate resources in the community.

3. Physical Group (Needs) may Include:

- Formal and informal contacts capable of assisting caregivers with daily tasks like lawn mowing, house repairs, grocery shopping, transportation, house-keeping, etc.

Family Members:

- Friends.
- Acquaintances.

Examples of Formal Contacts:

- Local community organisation providing support services for adult care centre and family caregiver respite.
- Church or faith-based groups.
- Tradespeople.
- Grocery stores offering delivery services.
- It is beneficial to have evaluations and to revise periodically.

REFERENCES

- Botswana Government Gazette Supplementary C, *Prevention of introduction or spread of Covid-19*, 2022.
- Botswana Qualifications Authority *Manual for development, validation, and registration of qualifications on the national credit and qualifications framework*, 2021, Version 2 page 9
- Bursack, C, *Strategies for getting organised while caregiving*, www.agingcare.com/articles/strategiesforgettingand-stayingorganisedwhilecaregiving, May 2020.
- Given, B, Sherwood, P, Given, C, *Knowledge and skills for caregivers*, American Journal of Nursing, (108), 9, 28-34, 2008.
- Huntsberry-Lett, A, *Making a care plan for elderly*, AgingCare.com, updated 2022.
- Katz, S, *Assessing self-maintenance: Activities of daily living, mobility, and instrumental activities of daily living*, Gerontology Society of America (31) (12), 721-7, 1983.

Scherbring, M, *Effect of caregiver perception of preparedness on burden in an oncology population*, Oncology Forum, 29 (6), 70-76, 2002.

BIBLIOGRAPHY

Asch, I, Prins, M, Willemse, I, *Development of an e-learning for caregivers to manage challenging behaviour of people with dementia*, The Gerontologist, Vol 56 (Supplement 3), pg 538, 2016.

Botswana Government Gazette Supplementary C, *Prevention of introduction or spread of Covid-19*, 2022.

Botswana Qualifications Authority *Manual for development, validation, and registration of qualifications on the national credit and qualifications framework*, 2021, Version 2 page 9.

Bursack, C, *Strategies for getting organised while caregiving*, www.agingcare.com/articles/strategiesforgettingand-stayingorganisedwhilecaregiving, May 2020.

Cameron, JI, Elliot, T, *Studying long term caregiver health outcomes with methodological rigor*, Neurology 84(13), 1292- 1293, 2015.

Duggleby, W, Ploeg, J, McAiney, C, Fisher, K, Ruiz, J, Ghosh, S, Peacock, S, Reid, M, Triscott, J, William, A, Swinndle, J, *A comparison of user and non-users of a web-based intervention for carers of older persons with Alzheimer's disease and related dementias: Mixed methods secondary*

analysis, Journal of Medical Internet Research 21(10), 14254, 2019.

Elliot, T, Pezent, G, *Family caregivers of older persons in rehabilitation*, Neuro Rehabilitation, (23), 439-446, 2008.

Ettxerberria, I, Salaberria, K, Gorostiaga, A, *Online support for caregivers of people with dementia: A systematic review and meta-analysis of RTCs and quasi-experimental studies*, Aging Mental Health, 1- 16, 2020.

Given, B, Sherwood, P, Given, C, *Knowledge and skills for caregivers*, American Journal of Nursing, (108), 9, 28-34, 2008.

Gatley, M, Ladin, K, *Family and other caregivers*, Chronic illness care, 11-118, 2018.

Hess, D, *Myths and systems: A response to "He needs to talk! A chaplain's case study of nonreligious spiritual care"*, Journal of Health Care Chaplaincy, 22(1), 17- 27, 2016.

Katz, S, *Assessing self-maintenance: Activities of daily living, mobility, and instrumental activities of daily living*, Gerontology Society of America (31) (12), 721-7, 1983.

Linderman, D, Kim, K, Gladstone, C, Apsoa-Veraro, E, *Technology and caregiving: Emerging interventions*

and directions for research, The Gerontologist, 60 (supplementary 1), 541-549, 2020.

O'Brien, T, *Multiple sclerosis, stressors and coping and spousal caregivers*, Journal of Community Nursing, (19), 123-135, 1993.

Rathnayake, S, Moyle, W, Jones, C, Colleja, P, *Family carers' needs related to management of functional disability in dementia care and use of health applications in information seeking: An online survey*, Collegian 27 (3), 288-297, 2020.

Rider, B, Lier, S, Johnson, T, Hu, D, *Interactive web-based learning: Translating health policy into improved diabetes care*, American Journal of Preventive Medicine, 50 (1), 122-124, 2016.

Scherbring, M, *Effect of caregiver perception of preparedness on burden in an oncology population*, Oncology Forum, 29 (6), 70-76, 2002.

Shata, Z, Amin, M, Kady, H, Mervat, W, *Efficacy of a multi-component psychosocial intervention programme for caregivers of persons living with neurocognitive disorders, Alexandria, Egypt: A randomized controlled trial*, Avicenna Journal of Medicine 7(2) 54-63, 2017.

Sullivan, AB, Miller, D, *Who is taking care of the caregiver?*
Journal of Patient Experience, 2(1), 7-12, 2015.

Xenakis, N, *Creating a professional development platform to
transform social work clinical practice in health*, *Social
Work in Health Care*, 57(6), 440-464, 2018.

Zimmerman, S, Sloane, P, Ward, K, Reed, D, Robbi, C,
Beeber, A, Gwyther, L, Matcher, M, *Helping dementia
caregivers manage medical problems: Benefits of an
educational resource*, *American Journal of Alzheimer's
Disease and other Dementias* 33(3), 173-188, 2018.

